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TITLE: CardioZen - Renal Safety Addendum to SAFETY_DOC_007
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RENAL SAFETY ADDENDUM

Reference: SAFETY_DOC_010
Date of issue: 12 August 2026
Publisher: Regional Medicines Safety Committee
Distribution: All prescribing sites; renal services; clinical review
Supersedes: nothing. This addendum is additive to SAFETY_DOC_007.

| SYNTHETIC DOCUMENT - NOT REAL MEDICAL GUIDANCE. Fabricated for a
| software demonstration. CardioZen is an invented medication. Nothing here
| describes a real product or should inform any clinical decision.

1. Purpose of this addendum

SAFETY_DOC_007 established potassium surveillance for patients receiving CardioZen who have chronic kidney disease. That requirement is unchanged and remains in force. This addendum adds a second, separate requirement arising from the Committee's July review and does not alter the first.

2. Background

Pooled dispensing and laboratory data from the participating sites covering nineteen months were reviewed. Among patients with chronic kidney disease established on CardioZen, the Committee observed a gradual decline in estimated glomerular filtration that was not detectable from potassium measurement alone, because potassium remained within reference limits in the majority of those patients until the decline was well advanced. Reliance on a single analyte was therefore judged insufficient for this population.

3. Monitoring requirement

Patients receiving CardioZen who have chronic kidney disease should have serum creatinine measured every six weeks for as long as treatment continues. This requirement takes effect from the date of this addendum. It applies in addition to, and not instead of, the potassium surveillance set out in SAFETY_DOC_007 section 3.

Where a site cannot meet the six-week interval from existing capacity, the site pharmacy lead should escalate to the Committee rather than allow the interval to extend.

4. Matters expressly not covered

The Committee considered and rejected a proposal to require routine

measurement of cystatin C. Cystatin C is not required. The Committee also noted that baseline renal function is commonly recorded at first prescription; that practice is encouraged but is not the subject of any interval requirement.

5. Contact

Queries relating to this addendum should be directed to the Committee secretariat through the usual site channel.